

UHB/BWH Urgent Suspected Uterine / Endometrial Cancer Referral Form

All USC Referrals should be made through the NHS eReferral Service (eRS).

THIS FORM SHOULD NOT BE USED IF THE PATIENT HAS HAD A HYSTERECTOMY

Please ensure to complete all appropriate boxes to aid clinical triage and appropriate care of the patient

PATIENT DEMOGRAPHICS		Date of Decision to refer:	
Surname: Surname		Address: Home Full Address (single line)	
First name: Given Name			
Gender: Male ☐ Female ☐			
NHS number: NHS NUMBER		Telephone Day Time: Patient Home Telephone Evening: Patient Mobile Telephone	
Date of birth: Date of Birth			
Interpreter required: Yes ☐ No ☐		Main Language: Main Language	

REASONABLE ADJUSTMENTS

Details: Single Code Entry: Requires reasonable adjustment for health and care access (Equality Act 2010)...

GP/REFERRER DETAILS

Referrer: Free Text Prompt	Practice Name: Organisation Name
Referrer's Role:	Practice Address: Organisation Full Address (single line)
Usual GP: Usual GP Full Name	
Organisation Code: Organisation National Practice Code	
Practice Telephone: Organisation Telephone Number	
Practice email: Organisation E-mail Address	

I have provided the patient with a 2 week wait information leaflet (leaflets available to order CRUK) Your urgent suspected cancer referral - text version (downloadable PDF) Publications (cancerresearchuk.org)	Yes ☐ No ☐
Is patient available in the next two weeks:	Yes ☐ No ☐

Please note, if a patient doesn't meet any of the below Reasons for Referral then this is **NOT** an Urgent Suspected Cancer Referral, please see **Urgent Pelvic US Scan Request form for unscheduled bleeding on HRT**

REASON FOR REFERRAL	TICK
1. Postmenopausal bleeding (bleeding in a person who has had amenorrhoea $\geq$ 12 months) not on HRT (this does not require risk stratification)	☐
2. Abnormal persistent vaginal bleeding for at least 3 cycles and $\geq$ 45 years not on HRT with BMI $\geq$ 40 OR h/o Lynch/Cowden syndrome OR three of these four criteria BMI 30-39, PCOS, diabetes, Nulliparous	☐
3. Unscheduled bleeding on HRT with 1 major OR 3 minor risk factors Major Risk Factors: ☐ BMI $\geq$ 40 ☐ Genetic Predisposition for Lynch Syndrome Major HRT Related Risk Factors: ☐ Sequential HRT use for > 5 years when started in women $\geq$ 45 years old ☐ $\geq$ 12 months of a lower progesterone dose in sequential HRT (*see below for clarification) ☐ > 6 months unopposed oestrogen Minor Risk Factors: ☐ BMI 30 – 39 ☐ PCOS ☐ Diabetes Minor HRT Related Risk Factors: ☐ 6 – 12 months of insufficient progesterone dosage in sequential HRT (*see below for clarification) ☐ > 12 months insufficient progesterone dose for oestrogen dose (**see dose table at the end of the form) ☐ 3 – 6 months unopposed oestrogen *More than 5 years of (Mirena or similar) in situ / Norethisterone or medroxyprogesterone acetate	☐

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(Provera) for <10 days per month/ Micronised progesterone (e.g. Utrogestan) for <12 days per month/ Tricycling HRT (e.g. Tridestra quarterly progestogen).		
Please Note: Minimum data set of 1 major or 3 minor risk factors must be included for referral to be accepted.		
4. Women on HRT with persistent unscheduled bleeding with a normal USS result after 6 months of HRT modification (two attempts at dose modification at 3 monthly intervals)		☐
Please Note: If persistent bleeding on HRT after one attempt at dose modification is an indication for an ultrasound scan (non-cancer pathway)		
5. Women who do not meet criteria for a Pelvic USS and have stopped HRT but continue to have persistent unscheduled bleeding at 4-week review		☐
6. Asymptomatic women with incidental finding of thickened endometrium on ultrasound scan (please categorise patient into appropriate risk and review endometrial thickness)		☐
Endometrial thickness >= 10 mm ☐		

***To aid clinical triage please arrange prior to referral: U&E's required to facilitate CT scan with contrast if necessary. If you have a scan report, please attach it to this referral. It will stop patient from having another scan appointment.**

U&Es and eGFR Result (within last 3 months)

☐ U&Es Results -	☐ Requested
☐ eGFR Results -	☐ Requested

Please detail any clinical findings and relevant clinical information in this section (including any recent investigations).

Please indicate whether the patient has had a hysterectomy	Yes ☐ No ☐
If referral for PMB, has patient been previously investigated for this within last 6 months	Yes ☐ No ☐
Body Mass Index (in the last year)	
>30 ☐ 30 – 39 ☐ < 39 ☐ Exceeds weighing scale limit	
I have performed a gynaecological examination and this is normal (if examination suspicious, referral should be under appropriate criteria on USC referral form)	Yes ☐ No ☐
Is patient willing to undergo a Transvaginal ultrasound scan	Yes ☐ No ☐
I confirm I have given the patient leaflet regarding management of postmenopausal bleeding	Yes ☐ No ☐

Please Patient Information for medication, past medical history, repeat medications, allergies, recent investigations. (from Clinical System)

Past Medical History – see below

Medications - see below

Allergies - see below

Recent Investigations - see below

ASSESSMENT OF PATIENT'S SUITABILITY FOR DIAGNOSTIC TESTS

(To aid clinical triage & reduce delay in secondary care)

Are there any concerns about the patient's mental capacity at this particular moment in time Yes ☐ No ☐

I have assessed frailty / begun best interest discussion: Yes ☐ No ☐ N/A ☐

Please sign the declaration below for mental capacity and / or frail / WHO performance status of 3 and 4. For these patients, please consider a best interest discussion, taking into consideration the need for intrusive intimate examination, further investigations and suitability for treatment, if cancer is diagnosed.

PERFORMANCE STATUS – W.H.O GRADE (To aid clinical triage)

Able to carry out all normal activities without restriction.	0	☐
Restricted in physically strenuous activity. Able to walk and do light work.	1	☐

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Able to walk and capable of all self-care. Unable to work. Up and about more than 50% of waking hours	2	☐
Capable of only limited self-care. Confined to bed or chair more than 50% of waking hours.	3	☐
Completely disabled. Cannot carry out any self-care. Totally confined to bed or chair.	4	☐

GP DECLARATION	
I confirm that I have discussed where appropriate based on the patient's mental capacity the possibility that the diagnosis may be cancer and that they may need to undergo a range of tests 'Please provide further details below if you are referring a patient with poor performance score or have concerns around patient capacity to consent to investigation'.	☐

Problems
Medication
Allergies
Investigations

Appendix 1: Licensed estrogen dose and proportionate progestogen dose

'The dose of the progestogen should be proportionate to the dose of estrogen. Women who require high dose estrogen intake should consider having their progestogen dose increased to ensure adequate endometrial protection.'

BMS 2022 Recommendation

Key: Prescribed estrogen dose for ultra-low, low, standard, moderate and high dose regimens

	Ultra-low dose	Low Dose	Standard dose	Moderate dose	High dose
Oestrogel	½ pump	1 pump	2 pumps	3 pumps	4 pumps
Sandrena	0.25 mg	0.5 mg	1 mg	1.5-2 mg	3 mg*
Lenzetto spray	1 spray	2 sprays	3 sprays	4-5 sprays*	6 sprays*
Patch	12.5 µg	25 µg	50 µg	75 µg	100 µg
Oral estradiol	0.5 mg	1 mg	2 mg	3 mg^	4 mg^

* Off-license use
mg = milligrams

^ Off-license use – rarely required to achieve symptom control
µg = micrograms

Progestogen dose per licensed estrogen dose in the baseline population

Estrogen dose	Micronised Progesterone continuous	Micronised Progesterone sequential	Medroxy progesterone continuous	Medroxy progesterone sequential	Norethisterone continuous	Norethisterone sequential	LNG-IUD (52mg)
Ultra/Low	100 mg	200 mg	2.5 mg	10 mg	5 mg*	5 mg*	One – for up to 5 years of use
Standard	100 mg	200 mg	2.5-5 mg	10 mg	5 mg*	5 mg*	
Moderate	100 mg	200 mg	5 mg	10 mg	5 mg	5 mg	
High	200 mg+	300 mg+	10 mg^	20 mg^	5 mg	5 mg	

*1 mg provides endometrial protection for ultra-low to standard dose estrogen but the lowest stand-alone dose currently available in the UK is 5 mg (off-license use of three noriday POP i.e 1.05 mg, could be considered if 5 mg is not tolerated).

^There is limited evidence in relation to optimal MPA dose with high dose estrogen; the advised dose is based on studies reporting 10 mg providing protection with up to moderate dose estrogen.

+There are limited evidence in relation to optimal micronised progesterone dose for moderate or high dose estrogen; until evidence is available to guide practice, the advised dose is based on studies reporting 100 mg/day providing protection with up to standard dose estrogen. If unscheduled bleeding occurs with ultra-low to moderate dose estrogen, and other progestogens are not acceptable, offer micronised progesterone at the dosage recommended for high dose estrogen.